Prodigy Anesthesia Quick Review

Hysterectomy

How is the patient positioned for an abdominal approach to hysterectomy?	Supine *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 821.*
What is the most frequent preoperative diagnosis for patients undergoing a vaginal hysterectomy?	Pelvic organ prolapse *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 820.*
List situations that limit the vaginal approach to hysterectomy.	Pelvic adhesions, uterine size, the pelvic bone design itself, the presence of gynecological cancers *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 820.*
How is the patient positioned for a vaginal hysterectomy?	Dorsal lithotomy position. The patient is positioned so that the perineum is at the end of the operating table, which maximizes surgical exposure. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 820.*
What are the advantages of a laparoscopic hysterectomy over an abdominal approach to hysterectomy?	Less pain, less incidence of wound complications, laparoscopy offers great visualization and exposure, recovery time is shorter, and bowel function returns more quickly. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 869.*
Why would a surgeon inject epinephrine or vasopressin into the cervix at the beginning of a vaginal hysterectomy?	To reduce perioperative bleeding of the vaginal cuff. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 821.*
What are common preoperative diagnoses for patients undergoing a vaginal or abdominal hysterectomy?	Pelvic organ prolapse, uterine myoma, dysmenorrhea/uncontrolled uterine bleeding, endometrial hyperplasia, pelvic pain (secondary to adhesions or endometriosis), gynecological cancers. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 820.*

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When placing a patient in stirrups for a vaginal hysterectomy, the stirrups should be well padded. This helps prevent injury to what nerve?

The common peroneal nerve

Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 821.

What must be present in order for a diagnosis of pelvic organ prolapse to be made?

One or more of the following must be present: uterine prolapse, cystocele, enterocele, urethrocele, or rectocele

Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 820.

What consideration should be made for a patient who has had a bowel prep and is presenting for a hystectomy?

The patient may be dehydrated and have electrolyte abnormalities.

Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 823.

Why is the operating room table placed in steep Trendelenburg during a laparoscopic hysterectomy?

So that the bowel falls away from the operative field.

Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 871.

How is the patient positioned for a laparoscopic hysterectomy?

Dorsal lithotomy position

Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 871.

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